

Covarrubias, Denise Susan

ED to Hosp-Admission
2/6/2026 - 2/9/2026 (3 days)
Status: Discharged
CTH CARSON TAHOE REGIONAL
MEDICAL CENTER

Last attending: Alexander Azzam, MD • Treatment team
Principal problem: Sepsis secondary to UTI (HCC)

Discharge Summary

Alexander Azzam, MD (Physician) • General Medicine

Discharge Summary

Admission Date: 2/6/2026 Discharge Date: 2/9/2026

Attending Physician: Alexander Azzam, MD Phone: 775-445-8795
Service: General Medicine

Discharge Diagnoses:

Assessment & Plan

Sepsis secondary to UTI (HCC)
Acute pyelonephritis
Gram-negative bacteremia

- E. Coli
Urine Culture >100,000 CFU/ml Escherichia coli !
Observations
Resulting Agency: CTH Lab

Susceptibility

	Escherichia coli		
	MIC		
Ampicillin	>=32 ug/ml	Resistant	
Ampicillin/Sulbactam	16 ug/ml	Intermediate	
Cefazolin	2 ug/ml	Sensitive	
Cefepime	<=0.12 ug/ml	Sensitive	
Ceftazidime	<=0.5 ug/ml	Sensitive	
Ceftriaxone	<=0.25 ug/ml	Sensitive	
Ciprofloxacin	1 ug/ml	Sensitive	
Ertapenem	<=0.12 ug/ml	Sensitive	
Gentamicin	<=1 ug/ml	Sensitive	
Levofloxacin	1 ug/ml	Sensitive	
Meropenem	<=0.25 ug/ml	Sensitive	
Nitrofurantoin	32 ug/ml	Sensitive	
Piperacillin/Tazobactam	<=4 ug/ml	Sensitive	
Trimethoprim/Sulfamethoxazole	<=20 ug/ml	Sensitive	

Type 2 diabetes mellitus

Hypertension

Asthma

Procedures performed:

None

Reason for Hospitalization:

Please see admission H&P from 2/6/2026 2:05 AM for full details.

In brief, Denise Susan Covarrubias is a 71 year old female with hx of DM, asthma, HLD who presented with fever and flank pain

Hospital Course:

Pt presented with above, found to have sepsis and urine frankly pyuric. Started on IV CTX for complicated UTI. Cultures in urine grew E.coli susceptible to CTX. Additionally pt had positive cultures with E. Coli but also likely contaminant as well. Pt improved on CTX alone. She will continue with ciprofloxacin to finish out her course of treatment.

Medications were reviewed. Plan was reviewed. Reasons to return for medical attention were reviewed. All questions were answered to the satisfaction of the patient and/or family.

Objective:

2/9/2026 8:22 AM		2/9/2026 5:28 AM		Body mass index is 28.86 kg/m².
Vital Signs		Last Height/Weight		
BP	169/79	Weight	73.9 kg	
Heart Rate	106			
Temp	36.2 °C (97.1 °F)			
Resp	16			
SpO2	92 %			

Physical Exam

Constitutional:

General: NAD

HENT:

Head: Normocephalic and atraumatic.

Eyes:

General: No scleral icterus.

Extraocular Movements: EOM normal.

Neck:

No tracheal deviation.

Cardiovascular:

Rate and Rhythm: Normal rate and regular rhythm.

Heart sounds: +S1/S2

Pulmonary:

Effort: Pulmonary effort is normal. No respiratory distress.

Breath sounds: diminished bases

Abdominal:

General: Bowel sounds are normal.

Palpations: Abdomen is soft.

Tenderness: There is no abdominal tenderness.

Musculoskeletal:

General: No tenderness or edema. No flank tenderness

Skin:

General: Skin is warm and dry.

Capillary Refill: Capillary refill takes less than 2 seconds.

Neurological:

Mental Status: alert and oriented to person, place, and time.

Psychiatric:

Behavior: Behavior normal.

Discharge Medications and Medication Changes

Medication List

START taking these medications

budesonide (inhalation) 1 MG/2ML suspension

Commonly known as: PULMICORT

Take 2 mL (1 mg) by nebulization every 6 hours as needed (SOB or wheezing). 1 box of vials

Notes to patient: Last given on 02/09/2026 at 06:55 AM

ciprofloxacin 500 MG tablet

Commonly known as: CIPRO

Take 1 tablet (500 mg) by mouth 2 times a day for 7 days.

CONTINUE taking these medications

albuterol 108 (90 Base) MCG/ACT inhaler

Commonly known as: PROVENTIL HFA; VENTOLIN HFA

INHALE 2 PUFFS BY MOUTH EVERY 4 HOURS AS NEEDED FOR SHORTNESS OF BREATH FOR RESCUE

amlodipine 5 mg tablet

Commonly known as: NORVASC

2.5 mg.

Notes to patient: Last given on 02/09/2026 @ 08:29 AM

insulin aspart 100 units/mL injection

Commonly known as: NovoLOG

Inject 6 Units subcutaneously 2 times a day.

Insulin Glargine-yfgn 100 UNIT/ML solution

INJECT 22 UNITS UNDER THE SKIN AT BEDTIME FOR DIABETES FOR DIABETES
(DISCARD OPENED VIAL AFTER 28 DAYS) **REPLACES LANTUS**

Notes to patient: Last given on 02/09/2026 @ 08:29 AM

mometasone 220 MCG/ACT aerosol powder, breath activated

Commonly known as: ASMANEX

Inhale 1 puff into the lungs once daily.

Where to Get Your Medications

These medications were sent to RENO VAMC
PHARMACY - RENO, NV - 975 KIRMAN AVENUE

975 KIRMAN AVENUE, RENO NV
89502

Phone: 855-339-0376

- budesonide (inhalation) 1 MG/2ML suspension
- ciprofloxacin 500 MG tablet

Upon review of home medications, for any controlled substances, opiates or narcotics, verbal consent was obtained for treatment after discussion about potential alternatives and risks/benefits of use of these medications. Education about appropriate use and dosing of medications as well as control substances was provided. Any controlled substances provided were done so in the setting of inadequate control with alternatives or no other appropriate alternatives.

Discharge Details:

Diet: as tolerated

Activity: as tolerated

Discharge Location: Home

Full Code

Post-discharge follow-up:

Study results still pending at time of discharge: none

PCP: No Assignment Pt Req

It required 32 minutes of time to evaluate patient, educate patient, review plans, and arrange discharge and follow up.

Author: Alexander Azzam, MD

Other Notes

[All notes](#)



H&P from Ryan Monti, MD (General Medicine)

Additional Orders and Documentation

 **Results**
Imaging
Microbiology

 **Meds**

 **Orders**

 **Flowsheets**

Encounter Info: [History](#), [Allergies](#), [Education](#), [Care Plan](#), [Detailed Report](#)

Communications

[View All Conversations on this Encounter](#)

Media

From this encounter

- [Document on 2/9/2026 1700 by Priscilla Henson, MD: ED Encounter Summary Bill](#)
- [Document on 2/6/2026 1728](#)
- [Document on 2/6/2026 0352](#)
- [Document on 2/6/2026 0349](#)
- [Document on 2/6/2026 0225](#)
- [Document on 2/6/2026 0225](#)
- [Document on 2/6/2026 0225](#)
- [Scan on 2/7/2026 1004 by Mayra Platero](#)
- [Scan on 2/10/2026 0831 by Holly Rutt](#)
- [Scan on 2/10/2026 0809 by Eric Van Beuge](#)
- [Scan on 2/6/2026 0236 by Leyda Salamanca](#)

Patient Unit Room/Bed	
CTH RMC SURG/ORTHO	0212/01

Hospital Problem List	
Urinary tract infection, site not specified	
◆ Sepsis secondary to UTI (HCC)	
Type 2 diabetes mellitus (HCC)	
Hypertension	
Acute pyelonephritis	
Asthma (HCC)	
Gram-negative bacteremia	

Care Timeline	
02/06	 Admitted from ED (Observation) 0744
	 Admitted 1124
02/09	 Discharged 1056

Discharge	
	Home or Self Care 

Patient Discharge Instructions [\(English Snapshot\)](#) - Printed 2/9/2026

Medication List at Discharge

-  [Albuterol Sulfate 108 \(90 Base\) MCG/ACT](#) INHALE 2 PUFFS BY MOUTH EVERY 4 HOURS AS NEEDED FOR SHORTNESS OF BREATH FOR RESCUE
-  [amlODIPine Besylate](#) 2.5 mg
- [Budesonide](#) 1 mg Nebulization Every 6 hours as needed, 1 box of vials
- [Ciprofloxacin HCl](#) 500 mg Oral 2 times daily
-  [Insulin Aspart](#) 6 Units Subcutaneous 2 times daily
-  [Insulin Glargine-yfgn 100 UNIT/ML](#) INJECT 22 UNITS UNDER THE SKIN AT BEDTIME FOR DIABETES FOR DIABETES (DISCARD OPENED VIAL AFTER 28 DAYS) **REPLACES LANTUS**
-  [Mometasone Furoate 220 MCG/ACT](#) 1 puff Inhalation Daily

MAR History by Date Range

All administrations
[02/06/26 - 02/09/26](#)